



PAWSIC

POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

www.pawsic.org

STANDARDS & ASSESSMENT FRAMEWORK

2026 EDITION

# Mobile Wound Group *Checklist*

A standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings.

## SECTION I

## Core Standards *All Settings*

Universal standards. Complete for every care setting.

### What's *Inside*

- ✓ **Operational Foundation** Parts 1 to 2
- ✓ **Clinical Practice** Parts 3 to 6
- ✓ **Compliance & Risk** Parts 7 to 9
- ✓ **Performance & Partnerships** Parts 10 to 13
- ✓ **Summary of Findings & Recommendations** Part 14

### Scoring

- 0 Absent
- 1 Emerging
- 2 Established
- 3 Best-in-Class

★ Critical items are flagged throughout to signal elevated regulatory or clinical risk.

### HOW TO USE

#### Complete Section I, then your *setting add-on*

Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV).

SECTION I  
Core

SECTION II  
SNF

SECTION III  
HHA

SECTION IV  
ALF/IL

## Purpose

The PAWSIC Mobile Wound Group (MWG) Checklist 2026 is a standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings. It is intended to support:

- Wound Providers (WPs) and Wound Provider Groups (WPGs): A tool to support program development or process improvement.
- Healthcare Leaders: A tool for any post-acute site of service to aid in the selection of a WP or WPG.
- Patients/Responsible Parties: A reference guide when selecting a WP or WPG.

## Intent

This checklist is solely a guide and covers a variety of competencies for both employed WPs and contracted WPGs. It is intended to be adaptable to the needs specific to a site of service, regulations, and applicable practice acts. Regulations, reimbursement structures, and policies that direct practice and care differ from state to state, payer to payer, and by site of service.

This checklist is not intended to replace your facility/organization's policies and procedures nor any local, state, or federal requirements. The utilization of this form is at your discretion. PAWSIC releases this form to your facility/organization to assist in selecting or guiding a partnership with a WP/WPG.

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**Instructions:** Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV). Section V is a patient-facing document; clinical reviewers do not need to complete it.

★ **Critical items are flagged throughout the document to signal elevated regulatory or clinical risk requiring priority follow-up**

## Assessment Intake

Complete this block before starting each evaluation. All fields are required for traceability.

| FIELD                                                                                       | ENTRY                                                                                                                                                                                                        | ADDITIONAL INFO |
|---------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------|
| Facility / Organization Name<br>(include prior company names, acquisitions, rebranding etc) |                                                                                                                                                                                                              |                 |
| WP / WPG Being Evaluated                                                                    |                                                                                                                                                                                                              |                 |
| Reason for Review: (New group, annual review, post-incident, due diligence, etc.)           |                                                                                                                                                                                                              |                 |
| WPG/WP patient volume/footprint<br>(number of facilities/states)                            |                                                                                                                                                                                                              |                 |
| Reviewer Name, Title/Role                                                                   |                                                                                                                                                                                                              |                 |
| Reviewer Disclosures (MWG competitor, vendor, former employee)                              |                                                                                                                                                                                                              |                 |
| Reviewer of Evidence Name, Title/Role                                                       |                                                                                                                                                                                                              |                 |
| Date of Assessment                                                                          |                                                                                                                                                                                                              |                 |
| Care Setting (select one)                                                                   | <input type="checkbox"/> SNF (Skilled Nursing Facility) <input type="checkbox"/> HHA (Home Health Agency)<br><input type="checkbox"/> ALF (Assisted Living) <input type="checkbox"/> IL (Independent Living) |                 |

### SECTION I

## Core Standards

Universal: Complete for all care settings

### SCORING

0 Absent

1 Emerging

2 Established

3 Best-in-Class

★ Critical Items, these need to be summarized separately

**Comments/Findings/Actions:** Reviewer to complete

**Evidence Reviewed:** in some cases this may be N/A; in others, the evidence used to support findings should be validated through observation of a legal document, website review, etc.

**CLUSTER A Operational Foundation**

Add: cybersecurity / HIPAA technical safeguards posture (MFA, device encryption, breach notification protocol). Should be flagged ★ Critical given mobile workforce on tablets in patient homes.

**PART 1 Organizational Structure & Credentialing**

| CRITERION / CONSIDERATION                                                       | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|---------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Years in operation                                                              |       |                               |                   |
| Ownership structure (private, PE-backed, hospital-affiliated)                   |       |                               |                   |
| State Licensure footprint; active vs lapsed status per provider                 |       |                               |                   |
| Ownership: a) Type (PE, Private, Hospital, etc.)                                |       |                               |                   |
| Ownership: b) Clinical Governance structure and independence                    |       |                               |                   |
| Settings currently served                                                       |       |                               |                   |
| Credentialing process (license verification, OIG/SAM, background checks)        |       |                               |                   |
| Payer participation and ability to align with facility contracts (ACOs, I-SNPs) |       |                               |                   |
| Malpractice coverage (limits, occurrence vs. claims-made)                       |       |                               |                   |
| Employment model (employed vs. contracted clinicians)                           |       |                               |                   |
| Corporate compliance program                                                    |       |                               |                   |
| Workforce policies addressing equitable hiring, training, and patient access    |       |                               |                   |
| Infection prevention requirements (TB, vaccinations, hand hygiene training)     |       |                               |                   |
| Identification standards (badging)                                              |       |                               |                   |
| Policy for addressing site-of-service concerns                                  |       |                               |                   |
| Training for Site Specific policies? If delegated to the site- who/when/how?    |       |                               |                   |
| Third-party accreditation (ACHC, CHAP, Joint Commission, etc.)                  |       |                               |                   |

**PART 2 Clinical Model & Scope of Services**

| CRITERION / CONSIDERATION                                                                                          | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|--------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Provider types, other clinical support, and level of MD oversight for NP models (MD, DO, NP, PA, PT, RN, MA, etc.) |       |                               |                   |
| Certifications (CWSP, CWS, WCC, WOCN, etc.; NCCA status)                                                           |       |                               |                   |
| Visit modality mix (in-person vs. allow telehealth)                                                                |       |                               |                   |
| On-site services: Debridement (types)                                                                              |       |                               |                   |
| On-site services: ABI / Doppler                                                                                    |       |                               |                   |
| On-site services: Biopsy                                                                                           |       |                               |                   |
| On-site services: Infection management                                                                             |       |                               |                   |
| On-site services: Application of cellular or acellular tissue-based products                                       |       |                               |                   |
| On-site services: Application and management of negative pressure wound management                                 |       |                               |                   |
| On-site services: ultrasound (ex: MIST)                                                                            |       |                               |                   |
| On-site services: Incision and drainage                                                                            |       |                               |                   |
| On-site services: Fluorescence / advanced tech for diagnostics                                                     |       |                               |                   |
| Telehealth capabilities and limitations (by whom, cost, app or equipment needed)                                   |       |                               |                   |
| After-hours clinical support                                                                                       |       |                               |                   |
| Equipment ownership (provider vs. facility)                                                                        |       |                               |                   |
| Supply model (kits, procedural supplies)                                                                           |       |                               |                   |
| Bedside vs. send-out testing                                                                                       |       |                               |                   |
| Percentage requiring external diagnostics                                                                          |       |                               |                   |
| Scheduling cadence & frequency for routine visits                                                                  |       |                               |                   |
| Scheduling for holidays, vacations, weather, unforeseen circumstances, etc.                                        |       |                               |                   |
| Management of routine provider coverage                                                                            |       |                               |                   |

| CRITERION / CONSIDERATION                                                                                                                                  | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Participation/attendance at Medical Staff Meetings                                                                                                         |       |                               |                   |
| Insurances accepted/not accepted; management of Hospice and uninsured patients                                                                             |       |                               |                   |
| Survey support- at what level does the MWG participate in survey, support during rounds/observations, and/or engage with surveyors to support the process? |       |                               |                   |

## CLUSTER B Clinical Practice

### PART 3 Clinical Practice Standards

*What "good care" looks like*

| CRITERION / CONSIDERATION                                                                         | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|---------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Standardized wound assessment methodology and re-assessment requirements                          |       |                               |                   |
| Etiology-first diagnostic framework                                                               |       |                               |                   |
| Care planning: Healing vs. maintenance vs. palliative goals                                       |       |                               |                   |
| Vascular Assessment (ABI or equivalent) performed and documented prior to compression orders      |       |                               |                   |
| Pain management protocols                                                                         |       |                               |                   |
| Documentation supporting wound etiology and type                                                  |       |                               |                   |
| Policy aligned with antibiotic stewardship: When/how to culture (Levine) vs. PCR test vs. neither |       |                               |                   |
| Management of atypical wounds / unclear etiology                                                  |       |                               |                   |
| Patient-centered decision-making (including palliative context)                                   |       |                               |                   |
| Clinical oversight structure (Medical Director / Lead WP)                                         |       |                               |                   |
| Internal review/audits/process improvement program for clinical care and outcomes                 |       |                               |                   |

**PART 4 Clinical Risk, Safety & Infection Control**

| CRITERION / CONSIDERATION                                                                                                                          | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|----------------------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Infection control practices (field-based and facility-based)                                                                                       |       |                               |                   |
| Disinfection of reusable equipment                                                                                                                 |       |                               |                   |
| Sharps handling and disposal protocol for in-home/in-facility debridement                                                                          |       |                               |                   |
| Pet/animal exposure protocol                                                                                                                       |       |                               |                   |
| Outbreak response support                                                                                                                          |       |                               |                   |
| Anticoagulation considerations for procedures                                                                                                      |       |                               |                   |
| Emergency escalation protocols                                                                                                                     |       |                               |                   |
| Management of clinical disagreements (e.g., staging disputes)                                                                                      |       |                               |                   |
| Peer review and provider performance management with adverse event reporting protocol (who, what threshold, what timeline) and a tracked event log |       |                               |                   |
| Consent obtained for procedures including imaging, sharp debridement, biopsy, and other                                                            |       |                               |                   |

**PART 5 Advanced Therapies & Utilization Management**

| CRITERION / CONSIDERATION                                                      | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|--------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Criteria for escalation beyond standard care                                   |       |                               |                   |
| Required prerequisites prior to escalation: Etiology addressed                 |       |                               |                   |
| Required prerequisites prior to escalation: Offloading/compression implemented |       |                               |                   |
| Required prerequisites prior to escalation: Infection controlled               |       |                               |                   |
| NPWT utilization criteria                                                      |       |                               |                   |
| Cellular/Tissue-Based Product (CTP) criteria                                   |       |                               |                   |
| Biophysical modalities use (Ultrasound, Pulsed Lavage, other)                  |       |                               |                   |
| Approval pathways for advanced therapies                                       |       |                               |                   |
| Debridement practices: Selective vs. surgical ratios                           |       |                               |                   |
| Debridement practices: Chemical, biologic, hydro-mechanical, and other         |       |                               |                   |

| CRITERION / CONSIDERATION                                                                                  | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Debridement practices specific toward end-of-life, palliative/hospice                                      |       |                               |                   |
| Provider incentives are disclosed: are clinicians' compensation tied to CTP volume?                        |       |                               |                   |
| Disclosure: Is CTP utilization rate used as a measurable metric (% of patients, \$/patient/month)?         |       |                               |                   |
| If CTP: Required prerequisites met and documented x 4 weeks prior to CTP application                       |       |                               |                   |
| Provider incentives are disclosed: are clinicians' compensation tied to debridement?                       |       |                               |                   |
| Provider incentives are disclosed: are clinicians' compensation tied to specific products or manufacturer? |       |                               |                   |

## PART 6 Clinical Integration & Operational Workflow

*How care actually functions in the environment*

| CRITERION / CONSIDERATION                                                                 | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|-------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Single point of contact for facility (dedicated care coordinator or equivalent)           |       |                               |                   |
| Order template alignment with the nursing workflow (zero re-keying expectation)           |       |                               |                   |
| Patient selection process                                                                 |       |                               |                   |
| Integration with interdisciplinary team (nursing, therapy, RD, SW, CNA)                   |       |                               |                   |
| Order workflows (writing, signing, turnaround expectations)                               |       |                               |                   |
| Communication: End-of-visit or routine reporting: what and when                           |       |                               |                   |
| Communication: Immediate reporting (change in condition or infection escalation)          |       |                               |                   |
| Communication: Notable issues identified outside scope of wound practice                  |       |                               |                   |
| Responsibility boundaries (provider vs. facility vs. agency)                              |       |                               |                   |
| Patient and family communication practices                                                |       |                               |                   |
| Telehealth between-visit support What is provider accessibility hours? Off-hours support? |       |                               |                   |

| CRITERION / CONSIDERATION                                                                                                                                                                      | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| EHR interoperability and access                                                                                                                                                                |       |                               |                   |
| EHR compatibility- how are the (1) date of wound/wound age/onset vs (2) date of MWG first visit differentiated? Is there a copy-paste function within MWG EHR and if so, when and what fields? |       |                               |                   |
| EHR Quality Review- are billing recommendations automated? If so, how are they generated? Does the provider approve and have ability to override?                                              |       |                               |                   |

### CLUSTER C Compliance & Risk

#### PART 7 Documentation & Data Integrity

| CRITERION / CONSIDERATION                                                                                                                                                                                                                                              | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Documentation platform (integrated vs. external)                                                                                                                                                                                                                       |       |                               |                   |
| Documentation: Same day note with signature                                                                                                                                                                                                                            |       |                               |                   |
| Imaging (photo) documentation: frequency, consent, storage, HIPPA/Privacy                                                                                                                                                                                              |       |                               |                   |
| Documentation consistency across settings and disciplines                                                                                                                                                                                                              |       |                               |                   |
| Internal audit: Wound type (diagnosis/etiology) accuracy                                                                                                                                                                                                               |       |                               |                   |
| Internal audit: Staging accuracy (PIs)                                                                                                                                                                                                                                 |       |                               |                   |
| Internal audit: Medical necessity monthly, on at least 10% of visits, with report and remediation pathway Ex: How does documentation should clearly support medical necessity, treatment goals, wound progress, and rationale for any continued debridement over time. |       |                               |                   |
| Alignment with facility documentation (MDS, OASIS where applicable)                                                                                                                                                                                                    |       |                               |                   |
| EMR data export capability for facility/agency (PDF chart copies, structured data, images/photos)                                                                                                                                                                      |       |                               |                   |
| Medicare Part B Dressing Supply documentation training                                                                                                                                                                                                                 |       |                               |                   |

**PART 8 Regulatory & Compliance Infrastructure**

| CRITERION / CONSIDERATION                                                                                                                                       | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Operationalization of LCD/NCD requirements                                                                                                                      |       |                               |                   |
| Compliance training (initial and ongoing)                                                                                                                       |       |                               |                   |
| Skin substitute / CTP utilization policy: includes criteria, prior authorization workflow, and provider-incentive alignment, facility notification and training |       |                               |                   |
| Anti-kickback / Stark exposure review: specifically around supply provision, equipment placement, and educational events                                        |       |                               |                   |
| Audit exposure/history (UPIC, TPE, RAC)                                                                                                                         |       |                               |                   |
| Telehealth billing compliance: place-of-service codes, modifiers, originating site rules.                                                                       |       |                               |                   |
| Medical necessity oversight processes                                                                                                                           |       |                               |                   |
| Authorization workflows                                                                                                                                         |       |                               |                   |
| Consent processes for procedures                                                                                                                                |       |                               |                   |
| Avoidable vs. unavoidable pressure injuries- how does the MWG provide support (ex: Lead or support the F686 CEP?)                                               |       |                               |                   |
| Skin Failure medical documentation and diagnosis- what medical framework is used to identify, document, and diagnosis this condition?                           |       |                               |                   |
| Patient-centered care policies                                                                                                                                  |       |                               |                   |
| Federal Conditions/Requirements of Participation (RoPs) for physician services in SNFs (initial visit timing, frequency, delegation rules, documentation)       |       |                               |                   |
| Resident rights framework: choice of attending physician, informed refusal, restraints, grievances, and how physician orders intersect with rights              |       |                               |                   |
| Photography and PHI: policies for wound imaging/photos (devices, storage, EMR integration, consent): HIPAA compliance                                           |       |                               |                   |

**PART 9 Governance, Escalation & Oversight**

| CRITERION / CONSIDERATION                          | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|----------------------------------------------------|-------|-------------------------------|-------------------|
| Medical Director qualifications and FTE allocation |       |                               |                   |

| CRITERION / CONSIDERATION                                                                                            | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|----------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Management of underperforming providers                                                                              |       |                               |                   |
| Instrument misuse / infection control violations                                                                     |       |                               |                   |
| Dispute resolution (e.g., staging disagreements)                                                                     |       |                               |                   |
| Internal peer review processes                                                                                       |       |                               |                   |
| Role during regulatory survey                                                                                        |       |                               |                   |
| Management of survey deficiencies involving WPs                                                                      |       |                               |                   |
| Process for offboarding a WP for performance or compliance reasons: and notification to affected facilities/agencies |       |                               |                   |
| Research participation governance (IRB, off-label use)                                                               |       |                               |                   |

## CLUSTER D Performance & Partnerships

### PART 10 Outcomes, Metrics & Performance Reporting

| CRITERION / CONSIDERATION                                                                                                                                   | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Healing rates by wound type (including palliative cases)                                                                                                    |       |                               |                   |
| Case-mix adjustment methodology disclosed                                                                                                                   |       |                               |                   |
| Third-party validation of reported outcomes                                                                                                                 |       |                               |                   |
| Benchmark source for healing rates cited                                                                                                                    |       |                               |                   |
| Time-to-improvement tracking                                                                                                                                |       |                               |                   |
| 30 Day hospital readmission rate attributable to wound-related causes (SNF benchmark of ~21% per Greenstein et al. 2024, and home health benchmark of ~15%) |       |                               |                   |
| Diabetic foot ulcer: amputation rate tracked                                                                                                                |       |                               |                   |
| Infection progression tracking                                                                                                                              |       |                               |                   |
| Recurrence rates                                                                                                                                            |       |                               |                   |
| % palliative vs. curative intent                                                                                                                            |       |                               |                   |
| Standardized reporting: % healed at 4 weeks                                                                                                                 |       |                               |                   |
| Standardized reporting: % healed at 12 weeks                                                                                                                |       |                               |                   |

| CRITERION / CONSIDERATION                              | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|--------------------------------------------------------|-------|-------------------------------|-------------------|
| Standardized reporting: % advanced therapy utilization |       |                               |                   |
| Standardized reporting: % debridement by type          |       |                               |                   |
| Frequency and audience of reporting                    |       |                               |                   |
| Data sharing for QAPI/QI                               |       |                               |                   |
| Formulary alignment vs. proprietary product use %      |       |                               |                   |

### PART 11 Products, Supplies & Cost Management

| CRITERION / CONSIDERATION                                                                                                             | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|---------------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| WPG ownership stake in any product or supplier company by any WPG principal or clinician                                              |       |                               |                   |
| Open Payments / Sunshine Act disclosures: aggregate payments received by WPG clinicians from manufacturers in the prior calendar year |       |                               |                   |
| Collaboration with Part B suppliers                                                                                                   |       |                               |                   |
| Policy on off-label/non-standard products                                                                                             |       |                               |                   |
| Product selection transparency                                                                                                        |       |                               |                   |
| DME alignment (NPWT, surfaces)                                                                                                        |       |                               |                   |
| Bring-your-own-supply practices                                                                                                       |       |                               |                   |
| Conflict of interest (COI) disclosures                                                                                                |       |                               |                   |

### PART 12 Education, Training & Competency

| CRITERION / CONSIDERATION                                                                                                     | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|-------------------------------------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Initial and ongoing clinician training programs                                                                               |       |                               |                   |
| On-boarding period with shadowing period prior to any independent/unsupervised visits with a minimum number of training hours |       |                               |                   |
| Training: Wound staging and etiology                                                                                          |       |                               |                   |
| Training: Unavoidable pressure injury framework                                                                               |       |                               |                   |
| Training: Palliative wound care                                                                                               |       |                               |                   |
| Training: Sharp debridement                                                                                                   |       |                               |                   |
| Training: Support surfaces and offloading                                                                                     |       |                               |                   |

| CRITERION / CONSIDERATION                                                                        | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|--------------------------------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Training: Compression                                                                            |       |                               |                   |
| Training: Chemical cauterization                                                                 |       |                               |                   |
| Training: Biopsy                                                                                 |       |                               |                   |
| Training: Incision and Drainage (I&D)                                                            |       |                               |                   |
| Education offered to facilities/agencies-structure, frequency and Stark compliance documentation |       |                               |                   |
| Competency validation with preceptor observation, written documentation- annual basis            |       |                               |                   |

### PART 13 Continuum of Care & Transitions

| CRITERION / CONSIDERATION                                                   | SCORE | COMMENTS / FINDINGS / ACTIONS | EVIDENCE REVIEWED |
|-----------------------------------------------------------------------------|-------|-------------------------------|-------------------|
| Discharge summaries                                                         |       |                               |                   |
| Ability to follow patients across settings                                  |       |                               |                   |
| Referral networks (outpatient, home health, hospice)                        |       |                               |                   |
| Coordination with receiving providers                                       |       |                               |                   |
| Orders for post-discharge care                                              |       |                               |                   |
| Billing practices and payer transparency                                    |       |                               |                   |
| Vascular, infectious disease, dermatology, endocrine, podiatry coordination |       |                               |                   |
| Post-healing surveillance / recurrence prevention program                   |       |                               |                   |

## PART 14

# Summary of Findings & Recommendations

Tally Met / Gap / N/A counts from each part above before completing this page.

## 14A Score Tally by Part

| SECTION | PART NAME                                    | # BEST-IN-CLASS | # ESTABLISHED | # EMERGING | # ABSENT | PRIORITY ACTIONS / NOTES |
|---------|----------------------------------------------|-----------------|---------------|------------|----------|--------------------------|
| Part 1  | Organizational Structure & Credentialing     |                 |               |            |          |                          |
| Part 2  | Clinical Model & Scope of Services           |                 |               |            |          |                          |
| Part 3  | Clinical Practice Standards                  |                 |               |            |          |                          |
| Part 4  | Clinical Risk, Safety & Infection Control    |                 |               |            |          |                          |
| Part 5  | Advanced Therapies & Utilization Management  |                 |               |            |          |                          |
| Part 6  | Clinical Integration & Operational Workflow  |                 |               |            |          |                          |
| Part 7  | Documentation & Data Integrity               |                 |               |            |          |                          |
| Part 8  | Regulatory & Compliance Infrastructure       |                 |               |            |          |                          |
| Part 9  | Governance, Escalation, & Oversight          |                 |               |            |          |                          |
| Part 10 | Outcomes, Metrics & Performance Reporting    |                 |               |            |          |                          |
| Part 11 | Products, Supplies & Cost Management         |                 |               |            |          |                          |
| Part 12 | Education, Training & Competency             |                 |               |            |          |                          |
| Part 13 | Continuum of Care & Transitions              |                 |               |            |          |                          |
| Add-On  | Setting-Specific Add-On (SNF / HHA / ALF/IL) |                 |               |            |          |                          |

**14B Strengths**

Areas where the WP/WPG demonstrates strong performance or exceeds standards.

| STRENGTHS |
|-----------|
|           |
|           |
|           |
|           |
|           |
|           |

**14C Gaps**

Specific criteria marked as "Gap" that require follow-up or corrective action.

| IDENTIFIED GAPS |
|-----------------|
|                 |
|                 |
|                 |
|                 |
|                 |
|                 |

**14D Risk Areas**

Items marked ★ Critical that remain unresolved and pose elevated regulatory or clinical risk.

| RISK AREAS |
|------------|
|            |
|            |
|            |
|            |

14E Recommendations

Specific recommended actions, responsible parties, and target timeframes.

| RECOMMENDATIONS |
|-----------------|
|                 |
|                 |
|                 |
|                 |
|                 |
|                 |
|                 |



Post-Acute Wound and Skin Integrity Council (PAWSIC) is a non-profit professional group advocating for best practices in wound care and patient-centered care in the post-acute medical sector.

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